



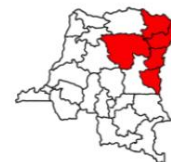
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°119/MVEBDB/10/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 10, 2026, a new province was affected, namely South Ubangi, specifically the Bulu health zone. Eighty new confirmed cases of Ebola Virus Disease (EVD), including 31 community deaths, were recorded in the provinces of Ituri (30 cases), North Kivu (43 cases), Haut-Uélé (6), and South Ubangi (1).

However, 18 intra-CTE deaths were also recorded.

ACCUMULATIVE CASE 7022	ACCUMULATIVE DEATH CONFIRMED 3,398	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 837	ACCUMULATIVE HEALED 1,671	TRACKING RATES CONTACTS (Of the Day) 87.6%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



62 out of 167
health zones affected
(37.1%)

Summary of key points from the last 24 hours

For the day of September 10, 2026, **80 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **31 community deaths**.

The confirmed cases are distributed across the provinces of **Ituri (30 cases)**, **North Kivu (43)**, **Haut-Uélé (6)** and **Sud Ubangi (1)**. In addition, **18 intra-CTE deaths** were recorded, including 12 in Ituri, 4 in Haut-Uélé and 2 in Nord-Kivu, bringing **the daily total of deaths to 49**.

In addition, **24 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (21 in Ituri and 3 in North Kivu).

A new province has been affected: South Ubangi, specifically the Bulu health zone, which has reported one confirmed case (death). The deceased was a 23-year-old male who had left South Kivu province on July 10, 2026.

From the start of the epidemic until September 10, 2026, **7,022 confirmed cases, including 3,398 deaths**, were recorded in the DRC, representing a case fatality rate of **48.4 %**.

The epicenter remains Ituri province, which accounts for **78.9%** of confirmed cases reported in the 7 affected provinces, while North Kivu accounts for **53.8%** % of new confirmed cases at the end of the day.

With the addition of the Bulu health zone in the South Ubangi province

The DRC, newly affected, currently has **62 health zones out of 167 (37.1%)** affected and **7 provinces impacted**. Ituri has 28 out of 36 health zones affected, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and Sud Ubangi (1/16), which has been added to the list.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 10, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	30	5,538	2,524	45.6%	28/36 (77.8%)
North Kivu	43	1,182	746	63.1%	16/34 (47.0%)
Haut-Uélé	6	270	114	42.2%	6/13 (46.1%)
Tshopo	0	24	9	37.5%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	4	3	75.0%	3/11 (27.3%)
South Ubangi	1	1	1	100.0%	1/16 (6.3%)
Total	80	7022	3,398	48.4%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 10, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5538	2524	45.6%	30	11	12	23
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	178	39	21.9%	1	0		0
Boga	2	2	100.0%				0
Bunia	1538	485	31.5%	10	8		8
Damascus	36	9	25.0%	3	0		0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	123	82	66.7%				0
Lita	256	125	48.8%	5	2		2
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	44	24	54.5%				0
Mangala	310	146	47.1%	1	0		0
Mongbwalu	649	295	45.5%	5	1		1
Nia-Nia	229	116	50.7%				0
Nizi	727	273	37.6%	1	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

Rwampara	1006	349	34.7%	2	0		0
Tchomia	65	25	38.5%	2	0		0
To ventilate *	N / A	427	N / A			12	12
North Kivu	1182	746	63.1%	43	19	2	21
Blessed	199	149	74.9%	4	3	1	4
Good	15	6	40.0%	5	2		2
Butembo	225	150	66.7%	5	1		1
Goma	1	0	0.0%				0
Kalunguta	24	11	45.8%	1	0		0
Katwa	499	320	64.1%	13	10	1	11
Kayna	1	0	0.0%				
Kyondo	29	17	58.6%	3	0		0
Lubero	10	4	40.0%				0
Mabalako	18	8	44.4%				0
Manguredjipa	8	3	37.5%	4	0		0
Masereka	15	6	40.0%				0
Musienene	110	58	52.7%	7	3		3
Mutwanga	3	2	66.7%				0
Oicha	11	7	63.6%	1	0		0
Vuhovi	14	5	35.7%				0
Haut-Uélé	270	114	42.2%	6	0	4	4
Boma	29	14	48.3%				0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	95	40	42.1%	3	0	2	2
Pawa	52	25	48.1%				0
Rungu	2	1	50.0%				0
Wamba	89	33	37.1%	3	0	2	2
Tshopo	24	9	37.5%	0	0	0	0
Bafwasende	4	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.00%				0
Viadana	1	0	0.00%				0
Ganga	2	2	100.00%				0
South Ubangi	1	1	100.0%	1	1	0	1
Bulu	1	1	100.0%	1	1	0	1
Total	7022	3398	48.4%	80	31	18	49

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province, as of September 10, 2026

DPS	Alerts Received		Total alerts	Alerts verified and		Alerts verified and		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
				validated (suspected case)	invalidated (no suspected cases)				
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	11	3	14	10	3	1	0	13	6
Bas-Uélé	26	1	27	4	1	21	0	5	4
Ituri	840	76	916	124	66	500	13	190	83
North Kivu	1,163	68	1231	148	68	833	0	216	98
South Kivu	ND	ND	ND	ND	ND	ND	ND	ND	ND
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	65	1	66	4	1	49	0	5	1
Total General	2,105	149	2,254	290	139	1,404	13	429	192

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

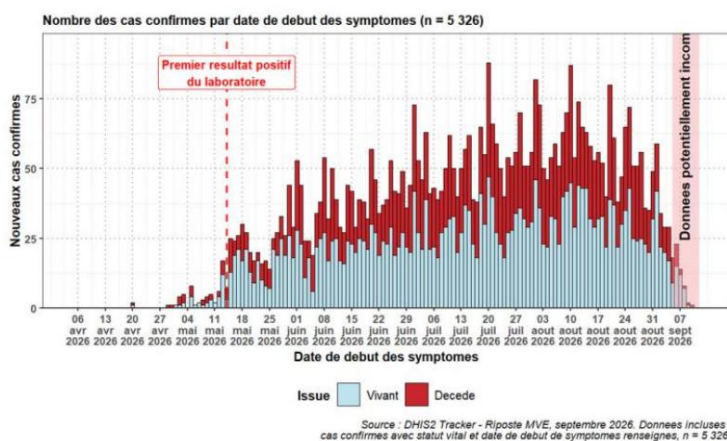
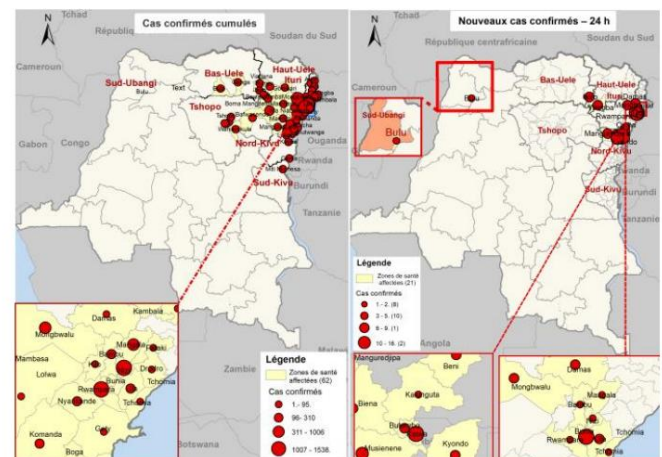


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

For the day of September 10, 2026, 2,254 alerts were reported in 5 provinces

Of the 7 currently affected, 1,846 (81.9%) could be verified. Thus, 429 alerts were validated as suspected cases. All of these were investigated (100%), with 192 transferred to CT/CTE (Table 3).

Of the 28,672 currently being monitored, 25,117 were seen, corresponding to a daily monitoring rate of **87.6%**, which is higher than both the 85% threshold and the proportion of the overnight (84.4%); 90.0% in Ituri (13,606/15,118), 86.7% in North Kivu (9,865/11,381), 77.6% in Bas-Uélé (104/134), 76.1% in Haut-Uélé (1,335/1,755) and 72.9% in Tshopo (207/284).

The listing of contacts around the first confirmed case is underway in the South Ubangi province.

1.1.3. Challenges

Contact tracing is suboptimal in 3 out of 5 provinces with ongoing contact tracing, with a significant decline in Tshopo province.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

During the course of September 10, 2026, only one alert was reported to the various Points of Emergency (PoE) and Points of Control (PoC) in North Kivu. **This** alert, concerning a live individual traveling from Ndando to Butembo, was confirmed after investigation at the Mususa PoC (Musienene Health Zone). The suspect was referred to the Kyambogho Health Center for treatment; two 3,000-liter tanks were also deployed there for the construction of two permanent handwashing stations. **In Haut-Uélé**, 84 infrared thermometers were received from the National Directorate of the National Humanitarian Program (PNHF) to strengthen screening and surveillance at PoEs and PoCs.

Table 4. Status of key PoE/PoC indicators by province, as of September 10, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	90% (54/60)	100% (19/19)	55.5% (5/9)	88.9% (16/18)	63.6% (7/11)	86.3% (101/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who switched to PoE/PoC % of	132,556	7,999	8,770	19,321	11,461	180,107
travelers screened	98.00% 99.00%	98.90%	98.20% 97.00%			98.10%
% of travelers who washed their hands	98.00% 99.00%	98.90%	98.00% 97.00%			98.00%
% of travelers made aware	99.00% 99.00%	100.00%	100.00% 100.00%	99.30%		
Number of alerts notified	0	1	0	0	0	1
Number of live alerts validated (suspected cases)	0	1	0	0	0	1
Number of bodies intercepted today; % of suspected	0	0	0	0	0	0
cases transferred to CT/CTE	N / A	100%	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	N / A	100%	N / A	N / A	N / A	100%

1.2.2. Challenges

Operationalization of PoE/PoC remains suboptimal: low reporting completeness persistent in South Kivu (55.5%, 5/9) and in Haut-Uélé (63.6%, 7/11: Dusu, Apodo, Aungba and Gossamu without internet connection); persistence of insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of the service providers of five PoCs (Nzebi, Galay, DGRPI Nizi, Base Yedi and Mabakese); in Tshopo, 18 PoCs activated out of the 30 planned; in South Kivu, absence of isolation device at the PoEs of Kavimvira, Kalundu Port SNCC and Baraka; absence of PoE/PoC report from Bas-Uélé.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 30 new positive results** (19 living and 11 deaths) out of 294 new samples received and analyzed (**positivity: 10.2%**).
- **North Kivu: 43 new positive results** (24 living and 19 deaths) out of 136 samples analyzed (**positivity of 31.6%**).
- **Haut-Uélé: 6 new positive results** (6 live) out of 236 samples analyzed (**positivity: 2.5%**).
- **South Ubangi: 1 new positive result** (1 death) in the sample analyzed (**positivity: 100.0%**).
- **Bas-Uélé: 3 samples** received and tested (3 live), **all were negative**.
- **Tshopo: 3 samples received and tested** (3 live), **all came back negative**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 25 of the 83 planned infection control rings were opened (30.1%), 238 of the 255 identified households were decontaminated (93.0%), and 5 of the 9 health facilities were decontaminated (55.0%); no infection prevention and control (IPC) kits were distributed to households. Regarding the health and safety surveys (HSS), 95 alerts were recorded (including 13 postponements from the previous day), 71 HSS were carried out (74.7%), and 54 bodies were swabed.
- **In North Kivu**, 28 rings were opened out of the 30 expected (93.0%), 10 households decontaminated out of 27 identified (37.0%) and 24 FOSA decontaminated out of 36 (66.6%); no household IPC kits were provided and 14 FOSA assessments were carried out with an average score of 37% in the health zones of Beni, Kalunguta, Biena and Musienene; risk assessment of staff exposure: 38/61 at high risk (Butembo, Biena and Beni); launch of training for 12 hygienists and 12 members of the EDS team in Butembo and a team of 12 members and 6 hygienists in Musienene with the support of the WHO; The EDS pillar received 54 alerts (19 reports from the previous day), carried out 46 EDS and swabbed 51 bodies out of 54 (3 failures at Kalunguta and Vuhovi).
- **In Tshopo**, decontamination of 3 rooms at the Bafwandambo Health Center and 9 rooms in 5 households of the 23rd and 24th confirmed cases (Bafwandambo Health Center, Bafwasende Health Zone) with the incineration of 4 mattresses; provision of infection prevention and control (IPC) supplies to the Bafwandambo Health Center and 10 households; monitoring and support of 15 community health workers (CHWs) with briefings for 159 service providers; 8 triage units installed out of 15 expected, and 500 liters of water supplied to Wanie-Rukula. The Health Determination and Surveillance (HDS) pillar recorded no alerts and conducted no HDS assessments in the last 24 hours.
- **In Haut-Uélé**, 6 rings were opened out of the 6 expected (100%), 7 households decontaminated out of 7 identified (100%) and 2 FOSA decontaminated out of 2 (100%), all of these actions being concentrated in the ZS of Pawa; 9 EDS alerts were recorded and 9 EDS carried out with swab (100%) in Isiro (6), Wamba (2) and Pawa (1).

1.4.2. Challenges

- **In Ituri** : low opening of rings (30.1%; 25/83); insufficient decontamination teams in Mambasa, Mangala and Komanda; lack of mobility in Mambasa, Bambu, Mandima and Mangala; insufficient PCI inputs in Mandima; 14 bodies not swabbed due to lack of inputs and teams in Fataki and Mandima.
- **In North Kivu** : low household decontamination rates (37.0%; 10/27); need to activate, train, and motivate decontamination teams in uncovered health zones (Biena, Masereka, Kyondo, Mabalako and Vuhovi); lack of traceability of some household cases

eligible for decontamination; lack of mobility for the deployment of provincial PCI teams in the field.

- **In Tshopo** : insufficient PCI inputs in households, schools, markets and churches; insufficient coverage of sorting circuits (53.3%; 8/15) and the incinerator at HGR Makiso-Kisangani remains out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 82 new admissions to the various dedicated facilities, compared to 104 discharges, including 21 recoveries. Thus, 435 patients are hospitalized out of 1005 beds, representing an occupancy rate of 43.3%; 399/413 (97%) patients received hot meals; 12/12 (100%) children are receiving LNPE/SLM at the CTE, 3/3 (100%) at the nursery, and 27/27 (100%) in the community.
- **In North Kivu**, 89 new admissions and 75 discharges were recorded, including 3 recoveries; 311 patients are hospitalized, representing an overall occupancy rate of oversaturation (141.4%); 49/49 patients (100%), 3/3 accompanying persons and 52/52 frontline providers received hot meals.
- **In Haut-Uélé**, there were 12 new admissions and 1 discharge (no recoveries). Thus, 64 patients are hospitalized out of 134 beds, representing an occupancy rate of 47.8%; moreover, 64 patients, 66 Accompanying persons and 28 PPLs benefited from three hot meals.
- **In Tshopo**, 1 new admission was recorded with no discharges (no recoveries). Nine (9) patients are in isolation for 31 beds, representing an occupancy rate of 29.0%.
- **In South Kivu**, no new admissions, 18 suspected cases remain in isolation (rate occupancy rate of 72.0%).

1.5.2. Challenges

Care still partially non-standard and limited capacities: **In Ituri**, the CTE/CTs are saturated on the side of the suspects: Nizi, Mambasa, Lolwa, Kilo, Mangala and Mandima; **In Haut-Uélé**, the Pawa Ebola Treatment Center (ETC) lacks sanitary facilities, and the Isiro center lacks space, water, and a constant supply of electricity. **In Tshopo**, the lack of safe blood and limited technical resources hinder patient care. **In Bas-Uélé**, the absence of ETCs/CTs and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,006 people have been vaccinated to date out of 11,000 targeted TPL/PPL (18,2 %), including 1,311 in Makiso-Kisangani, 301 in Kabondo, 287 in Mangobo, 89 in Tshopo and 18 in Lubunga; 2,500 doses were deployed in the health zones, with the balance settling at 494 doses at the zone level and 1,500 frozen doses at the Hub depot.
- **In Ituri**, training of vaccinators as part of preparations for the vaccination of PPL, which is scheduled to start on September 15, 2026 in Bunia

1.6.2. Challenges

Insufficient doses are available **in Tshopo** (4,500 received out of 11,000 requested), there is a lack of mobility for contact tracing (3 vehicles required for the mobile strategy), and the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, the stockout of ERVEBO in Buta has led to a temporary suspension of vaccinations for PPL and TPL.

1.7. Continuity of Care

- In **Tshopo**, two people with HIV arrived at the Bafwasende Health Zone referral hospital for preventive treatment; the Saint-Pierre Health Center was assessed, the recommendations of the GTM (Group for the Treatment of Immunization) were followed up, and essential generic medicines were delivered to the Lubunga Health Zone; 119 cases received experimental treatment, including 14 confirmed cases and 105 under post-exposure prophylaxis (68 treatments completed, 11 interrupted, and 26 ongoing). In Bas-Uélé, 12 contacts were monitored under compassionate prophylaxis in Buta and Dingila; the closure of the Rubi Lisanga Referral Health Center for 23 days disrupted priority activities in the Buta Health Zone.

1.8. Risk communication and community engagement

1.8.1. Main actions

- In **Ituri**, 24,627 home visits reached 76,958 people and 1,499 educational talks reached 19,657 people; 53 mass awareness campaigns reached 3,468 people, 8 community debates reached 564 people and 12 advocacy campaigns reached 48 people; 52 people were sensitized with the neighborhood chief of Rwankole (Bunia Health Zone) and a community dialogue was held with young people and church leaders from the Shari AS (Rwampara Health Zone); 158 community alerts were notified and 10 feedback issues were resolved within 48 hours.
- In **North Kivu**, 48,370 people were reached during 13,815 home visits, 2,167 people were reached during 45 educational talks and 21 mass communication actions; 394 community alerts were notified out of 450 expected (87.5%), 8 feedbacks were resolved out of 13 (61.5%) and 1,043 people were involved in community actions out of 4,750 (21.9%); monitoring of prevention measures in three schools in the city of Beni with UNICEF and EPST and mobilization of 6 group chiefs and 8 local chiefs of Manguredjipa.
- **Haut-Uélé** : 13 mass activities reached 3,994 people, 103 educational talks reached 525 people and 350 home visits reached 2,329 people; 11 community alerts were notified (Isiro 8, Pawa 2 and Boma Mangbetu 1) and preparations for the unloading of 17 survivors of Ebola virus disease continue in Pawa, Wamba and Gombari.
- In **Tshopo**, 5 mass awareness campaigns reached 1,506 people in the 5 health zones of the city of Kisangani, 2 educational talks reached 80 people in Bafwasende and Mangobo and 284 home visits reached 1,589 people; 3 radio programs were broadcast and 12 spots were broadcast in 6 press outlets; 23 community alerts were notified as well as 2 feedbacks, all resolved; official launch of the CTE Météo by His Excellency the Minister of Public Health.

1.8.2. Challenges

Persistent community resistance: In Tshopo, rumors of an "Ebola business" and demand for the vaccine among people living with HIV fueled two infodemics that were resolved within 48 hours, and one confirmed case who escaped from the Ebola Treatment Center remains at home. In Haut-Uélé, there is a complete breakdown of visibility tools and communication equipment in all health zones, and service providers have not been paid for over two months. In Ituri, resistance to Ebola Health Services (EHS) persists in Nyankunde, Nia-Nia, Mambasa, and Rwampara. In North Kivu, a family in Munzambaye (Butembo health zone) refuses decontamination of their household, and community support remains fragile in Kalunguta and Vuhovi.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- In **Ituri**, 22 of the newly confirmed cases and 29 of the 74 suspected cases received SMSPS interventions; 76 results were announced, including 22 positive results, 51 PPLs supported and 89 households assisted, 31 children and 42 recoveries monitored in the community.

- **In North Kivu**, 27 people among the new confirmed cases and 187 confirmed cases under follow-up were taken care of, as well as 115 new suspects; 145 results announced, including 27 positive ones, 69 PPL supported, 67 households accompanied, 19 of the 19 cured, i.e. 100% followed up; 467 people (186 men, 203 women and 78 children) benefited from psychoeducation sessions.
- **In Tshopo**, 2 confirmed cases, 2 new suspected cases and 1 suspect under follow-up benefited from SMSPS interventions; 6 contacts were supported at the CT of the Makiso-Kisangani General Referral Hospital and 216 people benefited from psychoeducation, including 186 volunteer frontline first responders from Makiso-Kisangani and 30 members of the community from the Kabondo Health Zone.
- **In Haut-Uélé**, 6 confirmed cases under monitoring, 2 new suspected cases and 4 suspected cases under monitoring received SMSPS interventions, as well as 35 caregivers at the Ebola Treatment Center (ETC) and 21 people with HIV/AIDS (PPL); 14 former cases were monitored and 5 recovered patients from the Wamba ETC are awaiting discharge.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions around the latest confirmed cases (Mangala and Komanda). **In North Kivu**, low coverage in the health zones of Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi; lack of nurseries in 10 affected health zones; insufficient human resources and motivation in 10 health zones. **In Haut-Uélé**, insufficient nutritional kits for weaned infants and difficulties with the mobility of community health workers. **In Tshopo**, absence of SMSPS providers in Wanie-Rukula, Bafwasende, and Isangi.

1.10. Logistics

1.10.1. Main actions

- **In North Kivu**, 27 calls were received, resulting in 27 successful dispatches (100%), and 9 ambulances were mobilized out of 11 available (82%); 49 patients (31 suspected and 18 confirmed), 3 caregivers, and 52 frontline workers received a hot meal (100%); 1 orphaned infant is receiving breast milk substitute, and 2 separated children are being cared for at the daycare center.
- **In Tshopo**, the Meteorological Treatment Center (CTE Météo) was handed over by the logistics team to the support pillar, followed by monitoring of the construction of the Bafwasende CTE, finalization of the CTMI rehabilitation plan with the National Institute of Public Health (INSP), and support for the mobility of the IPC, support, communication, PoE/PoC, and vaccination pillars. In Bas-Uélé, the surveillance team was deployed and packages were tracked with air services; the available chlorine stock is 50 kg.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, no data has been transmitted by the security pillar in the last 24 hours).

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri, 34

response staff members (7 men and 27 women) received the PSEA briefing and were sensitized; 177 people (83 men and 94 women, including one 9

People living with disabilities were sensitized in the community, and three partners (WHO, IMC, and Save the Children) reported on their PSEA activities. In Bas-Uélé, a community consultation with 65 participants was held in Koteli, and an 11-member complaints management committee was established, including 9 women and 2 focal points.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Performance remains consolidated in epidemiological investigations (**100% of suspected cases investigated**) as well as a daily proportion of contact tracing (**87.6%**) higher than the previous day (84.4%) and above the threshold of 85.0%.
- A new province has been affected, namely South Ubangi (Bulu health zone); transmission is sustained in the provinces of Ituri, North Kivu, and Haut-Uélé, while Tshopo province is showing sporadic reports. North Kivu province accounts for more than half (**53.8%**) of the new confirmed cases reported in the last 24 hours.
- Thus, **80 new confirmed cases**, including **31 community deaths and 18 intra-CTE deaths**, were recorded in the last 24 hours, bringing the national total to **7,022 confirmed cases and 3,398 deaths (case fatality rate: 48.4%)**.
- Ituri remains the epicenter of the epidemic, concentrating **78.9%** of cumulative confirmed cases and **37.5%** of new cases reported in the last 24 hours, with North Kivu getting closer (**53.8%**).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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